

ChiroPillar

STRENGTH IN ALIGNMENT

Master Executive Summary

The Complete ChiroPillar Strategy Packet

Prepared for: Dr. Scott Wagner

Build & scale team: Eric Skeldon · Scott McGrath · Kingdom Broker

Confidential — Internal Working Document · June 2026

What's Inside

This packet brings together the full ChiroPillar thesis in one place — strategy, financials, product, and brand. It is organized in four parts:

Part 1 **Executive Summary — The ChiroPillar Ecosystem** — The big picture: the acquisition & partnership model (3–5x, 50/50 seller financing, 4% profit share, rollover equity, tax-efficient exit), the five revenue engines, the team, and the conference strategy.

Part 2 **Chiropractic Roll-Up Deep Dive** — The market, the deepest industry problems, the operating playbook behind the ~\$250K-per-office uplift, the valuation/multiple framework, and the worked roll-up model.

Part 3 **App & Telehealth Deep Dive** — The free → \$9/mo → \$74/yr product, AI/clinical evidence, monetization & MRR math, and exactly how insurance (RTM + telehealth) can cover it.

Part 4 **Brand & Identity Brief** — The ChiroPillar name, the mascots (Cap and the Atlas vertebra), color palette, do's & don'ts, and the clinic-signage system.

Scott — this is the operating blueprint. Parts 2–4 are the detailed support behind the one-page logic in Part 1. Everything here is built to be implemented: buy disciplined, improve each office, wire in the app and brand, and scale toward an institutional exit. Figures are planning benchmarks to confirm against real clinic financials and with legal/tax counsel.



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The ChiroPillar Ecosystem

A Full Chiropractic Roll-Up, Technology & Growth Platform — Executive Summary

Prepared for: Dr. Scott Wagner

Build & scale team: Eric Skeldon · Scott McGrath · Kingdom Broker

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Executive Summary

ChiroPillar is a full chiropractic roll-up and growth ecosystem: we acquire and partner with independent chiropractic offices, make them measurably more profitable with shared technology and systems, consolidate them under one national brand, and create wealth on three fronts at once — operating profit, multiple arbitrage, and recurring digital and consulting revenue. The model is structured so the chiropractors we partner with earn more than they did on their own, keep upside through equity and profit share, and exit tax-efficiently — while the platform compounds value toward an institutional sale.

The thesis rests on a simple, proven arbitrage. Independent chiropractic offices are bought at roughly 3–5x earnings. Inside a professionalized platform with \$5M+ of EBITDA, the same earnings are valued at 8–10x. We capture the spread twice: first by improving each clinic's profitability (the operating playbook and technology add a target of ~\$250K of EBITDA per mature office), then by re-rating every dollar of that improved EBITDA at the platform multiple. The chiropractic market is large (~\$8B), highly fragmented (no player above ~5% share, ~58% solo practitioners), and barely consolidated — an open, cheap, and timely runway.

What makes ChiroPillar different from a plain roll-up is the surrounding ecosystem. A free-to-start consumer app and AI/telehealth funnel feeds patients into the clinics at low cost; high-ticket consulting, masterminds, and conferences (a Gym Launch-style growth engine) monetize the broader profession and build the acquisition pipeline; and access to large doctor networks and the industry's biggest conferences gives the platform distribution most competitors lack. Dr. Scott Wagner leads the clinical systems, operating standards, and implementation; Eric Skeldon and Kingdom Broker build the technology, apps, funnels, and brand; Scott McGrath helps execute — recruiting and leading the A-player operating team that runs and scales the platform; and trust and tax structuring for exiting owners is handled with Nexsess and an LL.M. (tax) attorney.

In one line: buy chiropractic offices cheaply, make them more profitable and the owners wealthier, wrap them in technology, brand, and a growth machine, and sell the consolidated platform at a far higher multiple — a win for the chiropractors, the patients, and the platform.

The Acquisition & Partnership Model

We acquire or partner with offices on terms designed to be attractive to the selling chiropractor while preserving the platform's arbitrage:

- **Valuation:** purchase at a 3–5x multiple of clinic earnings (cheap at the office level).
- **Structure:** ~50% cash at close + ~50% seller financing (a seller note paid from clinic cash flow), reducing upfront capital and aligning the seller with the clinic's continued success.
- **Ongoing 4% profit share:** the partnering chiropractor receives a 4% profit share so that — combined with sale proceeds, note payments, and a lighter operating load — they earn more than they did running the office alone.

- **Rollover equity at a higher multiple:** sellers can roll a portion of their proceeds into equity in the ChiroPillar / Dr. Wagner platform, which is valued at 8–10x rather than their stand-alone 3–5x — a second, larger payday at the platform's exit.
- **Tax-efficient exit:** before they sell, owners work with Nexxess and an LL.M. (tax) attorney to set up trust and entity structures intended to reduce the tax owed on their exit proceeds.

Why the chiropractor comes out ahead (illustrative)

Example: an office producing \$250,000 of EBITDA.

Component	Illustrative value
Purchase multiple	4x → \$1.0M enterprise value
Cash at close (50%)	\$500,000
Seller note (50%, paid from cash flow)	\$500,000 + interest
EBITDA after ChiroPillar systems (+~\$250K)	~\$500,000
Ongoing 4% profit share	Recurring income on improved profits
Optional rollover equity → platform at 8–10x	Re-valued at 2x+ their stand-alone multiple
Operating burden	Lower — shared back office, billing, marketing

The package, not any single line, is the pitch: cash now, a secured note, a continuing profit share on a more profitable clinic, less day-to-day burden, tax-efficient structuring, and equity in a platform worth a multiple of their old business. That total is designed to exceed what they would earn continuing solo — which is what makes owners say yes.

The Full Ecosystem — Five Revenue Engines

ChiroPillar is built as layered, reinforcing revenue engines, from highest scale to highest value:

Engine	What it is
1. Clinic roll-up (core EBITDA + arbitrage)	Acquire offices at 3–5x, improve to higher EBITDA, sell platform at 8–10x.
2. App + AI/telehealth funnel	Free → \$9/mo → \$74/yr; low-cost patient acquisition + recurring MRR + reimbursable RTM/telehealth.
3. High-ticket consulting (Gym Launch style)	Coaching packages, masterminds, done-with-you systems for chiropractors — profit + acquisition pipeline.
4. Conferences & events	Sponsoring/booths at the largest chiropractic conferences; hosting ChiroPillar events for brand, leads, and deal flow.
5. Ecosystem services	Trust/tax setup, billing/RCM, marketing, technology, and group purchasing sold across the network.

Each engine feeds the others: consulting and conferences generate acquisition leads; the app lowers patient-acquisition cost for every clinic; the clinics give the app and consulting real-world credibility and outcomes data; and all of it compounds the EBITDA that gets re-rated at exit.

Team & Partners

- **Dr. Scott Wagner — owner / operator:** implements and leads the clinical systems, operating standards, and per-office improvement playbook; the platform consolidates under his company at the higher multiple.
- **Eric Skeldon — Kingdom Broker:** builds the technology, apps, funnels, brand, and deal structuring that power acquisition and recurring revenue.
- **Scott McGrath — execution & talent:** helps get it done; recruits and leads the A-player operating team to run and scale the platform.
- **Nexsess + LL.M. (tax) attorney:** provide trust and tax structuring so exiting chiropractors keep more of their proceeds.
- **Doctor networks:** access to some of the largest physician/chiropractor networks for referrals, recruiting, and distribution.

High-Ticket Consulting & Events (Growth Engine)

Modeled on the Gym Launch / high-ticket info-and-implementation playbook, this layer monetizes the wider profession and doubles as the acquisition pipeline:

- Tiered offers: low-cost workshop/challenge → core coaching program → high-ticket mastermind / done-with-you platform onboarding.
- Live events and conferences that build the ChiroPillar brand, generate qualified leads, and surface acquisition targets (the best-performing attendees become partners).
- Recurring software + services (app, billing, marketing) attach to every consulting client, turning education into long-term revenue.

Conference Sponsorship Strategy

Sponsoring and exhibiting at the largest chiropractic conferences puts ChiroPillar in front of thousands of owners — for brand, consulting leads, and acquisition conversations. Priority targets:

Event	2026 timing	Scale	Why it matters / booth fit
Parker Seminars — Las Vegas	Feb 26–28	World's largest; thousands of attendees, 64 speakers, 15 tracks (75th anniv.)	Flagship sponsorship — maximum reach & prestige.
The National by FCA — Orlando	Aug 27–30	3,300+ attendees, ~420-booth expo, 70+ speakers	Largest expo floor — best for a booth and demos.

Event	2026 timing	Scale	Why it matters / booth fit
California Jam (Cal Jam)	Spring (So. Cal.)	2,000+; fast-growing, high-energy	Practice-builder / entrepreneurial crowd — ideal for consulting offers.
CalChiro & state association events	Throughout 2026	Regional	Geo-targeted to clinic-acquisition markets.
Technique/philosophy seminars (e.g., DE, CBP)	Throughout 2026	Engaged niche audiences	Credibility with high-volume, systems-minded DCs.

Recommended play: anchor brand presence with a Parker Seminars sponsorship, run the primary lead-gen booth at The National by FCA (largest expo), and use Cal Jam to sell the high-ticket consulting offer — then concentrate follow-up where ChiroPillar clinics and acquisition targets are located.

Why It Wins

- **Owners win:** more money, less burden, equity upside, tax-efficient exit.
- **Patients win:** free app, better access, telehealth, and consistent quality care.
- **Platform wins:** cheap deal flow, multiple arbitrage, recurring revenue, and a brand + distribution moat the digital-only and clinic-only competitors can't match.
- **Timing wins:** chiropractic is where dental/vet were before consolidation — early, fragmented, and cheap.

Immediate Next Steps

1. Stand up the MSO/PC and trust/tax structure with counsel (Nexsess + LL.M. attorney).
2. Acquire 1–2 anchor offices on the 3–5x, 50/50, 4% profit-share terms; prove the per-office uplift.
3. Launch the MVP app + funnel in anchor markets; begin RTM/telehealth billing through providers.
4. Book Parker Seminars and The National by FCA presence; build the high-ticket consulting offer.
5. Build the acquisition pipeline via consulting/conferences; scale toward \$5M+ platform EBITDA and an institutional exit.

Sources & Disclaimers

- [Parker Seminars Las Vegas 2026 — world's largest chiropractic event \(GlobeNewswire\)](#)
- [The National by FCA — overview \(3,300+ attendees, 420-booth expo\)](#)
- [The National by FCA 2026 — Orlando, Aug 27-30](#)
- [California Jam \(Cal Jam\) — Chiropractic Economics](#)
- [Parker Seminars — official site](#)

This summary builds on the companion ChiroPillar documents (Roll-Up Deep Dive, App & Telehealth Deep Dive, Brand Brief). Deal terms (3-5x multiples, 50/50 seller financing, 4% profit share) and the seller example are illustrative and must be confirmed against each target's actual financials. Trust/tax structures depend on individual circumstances and require qualified legal and tax counsel. Healthcare ownership requires MSO/PC structuring and compliance review. This is not legal, tax, financial, or investment advice.

Chiropractic Roll-Up

Market, Deepest Problems, Operating Solutions & Valuation Deep Dive

Prepared for: Eric & Dr. Wagner

Purpose: Build the strategic + financial thesis for acquiring, scaling, and rolling up chiropractic offices into a sellable platform.

Date: June 2026

Status: *Internal working document — figures are industry benchmarks to be confirmed against Dr. Wagner's actual P&Ls.*

Contents

1. Executive Summary

Chiropractic is one of the last large, highly fragmented, cash-friendly healthcare verticals that has not yet been consolidated the way dental, dermatology, vet, and physical therapy already have. That is precisely why it is attractive. The thesis behind Dr. Wagner's plan rests on three legs that all have to be true at once, and the evidence says they are:

- **Fragmentation = supply of cheap targets.** The U.S. has roughly 70,000 licensed chiropractors, ~35 million patients a year, and no single company holding even a 5% share. Over half of all practices are still solo operators. Small practices change hands at only ~3x EBITDA (or ~1.75–2.3x seller's discretionary earnings).
- **Operational upside = the "\$250K per office" claim is credible.** Typical independent clinics run 55% + overhead and leave large amounts of money on the table in billing, membership conversion, ancillary services, and pricing. These are exactly the levers a centralized operator pulls — and \$150K–\$300K of incremental EBITDA per mature office is a realistic target, not hype.
- **Multiple arbitrage = the real wealth creation.** Buy individual offices at 3–5x, professionalize and bundle them, and a platform doing \$5M+ of EBITDA sells to private equity or a strategic at 8–10x. You earn money twice: once on the operational uplift, and again on the re-rating of every dollar of EBITDA as the platform scales.

Combine the three and the math is compelling: a dollar of EBITDA bought for ~\$4 at the office level, grown to ~\$2 of EBITDA through operations, and ultimately valued at ~\$9 inside a platform, can produce a 4–6x return on invested equity over a typical hold — before any leverage. This report lays out the market, the consolidators already moving, the deepest structural problems, the operating playbook that unlocks the per-office uplift, and the valuation framework that determines what scale you must hit to command the top multiple.

The one-paragraph thesis

Acquire 8–15 owner-operated chiropractic offices in contiguous markets at 3–5x EBITDA; install a shared management services organization (MSO) that standardizes billing, membership/recurring-revenue conversion, ancillary cash services, marketing, and back-office; lift each office's EBITDA by ~\$250K; assemble a platform clearing \$5M+ in normalized EBITDA with clean financials and a repeatable de novo + acquisition engine; and exit to a larger healthcare consolidator or PE sponsor at 8–10x within 4–6 years.

2. The Market: Big, Fragmented, and Under-Consolidated

2.1 Size and growth

The U.S. chiropractic services market is estimated at roughly \$8 billion in annual revenue (2024), serving more than 35 million patients per year. Estimates vary widely by methodology — narrower "clinic services" definitions run lower, while broader musculoskeletal-care definitions run higher — but every credible source agrees on the direction: steady mid-single-digit to double-digit growth driven by an

aging population, the shift toward non-opioid and conservative pain management, and rising consumer willingness to pay cash for wellness.

There are approximately 70,000 licensed chiropractors in the U.S. (around 38,000 actively employed per BLS counts). Patient demand is durable and recurring by nature — chiropractic care is episodic-to-continuous, which is what makes membership and care-plan models so powerful.

2.2 Fragmentation — the core of the opportunity

This is the single most important fact for a roll-up: the industry is extraordinarily fragmented, with no company holding more than ~5% market share. Roughly 58% of providers operate as solo practitioners and only ~42% sit inside any kind of group or multi-provider setup. Only an estimated 15–20% of the market is “corporately aligned” today.

That fragmentation is a direct consequence of the profession’s history: chiropractic was built around the independent, owner-operator model, with clinicians who are excellent at care delivery but typically under-equipped on billing, marketing, pricing, and back-office scale. The result is thousands of sub-scale, sub-optimized offices — a deep, cheap acquisition pipeline for an operator who can bring professional management.

Market characteristic	Figure / status	Roll-up implication
Annual market size (US)	~\$8B (2024 est.)	Large enough to build a national platform
Licensed chiropractors	~70,000	Deep operator + target pool
Patients per year	35M+	Durable, recurring demand
Solo practitioners	~58% of providers	Abundant single-office targets at low multiples
Largest player share	<5%	No incumbent has locked up the market
Corporately aligned	~15–20% today	Early innings vs. dental/vet (much higher)

Why this matters: Dental and veterinary care were similarly fragmented 10–15 years ago. Consolidators that moved early bought practices cheaply and rode both operational improvement and multiple expansion. Chiropractic is roughly where dental was before DSOs scaled — the window for early, well-priced acquisitions is open but will not stay open forever.

3. Who Is Rolling These Up — The Consolidation Wave

Consolidation in chiropractic is real but still early, which is good news for a new entrant. The landscape breaks into four camps: the public franchisor, PE-backed corporate clinic groups (MSOs), franchise networks, and — increasingly — insurer-aligned musculoskeletal (MSK) networks. Below are the names actually moving capital.

3.1 The public benchmark: The Joint Chiropractic (NASDAQ: JYNT)

The Joint is the clearest public comp and the proof-of-concept for a scaled, cash-based chiropractic model. It is a retail, membership-driven, insurance-free concept (average adjustment ~\$37, ~85%+ of revenue from memberships).

The Joint Corp — FY2025	Figure
Clinics (end of 2025)	960 (885 franchised)
System-wide sales	\$532.4M
Company revenue	\$54.9M
Net income	\$2.9M
Adjusted EBITDA (consolidated)	\$13.0M (+13.9% YoY)
Patient visits (2025)	14.4M+
Avg visits/clinic/week	304 (median 279)
Franchise economics	7% royalty + 2% marketing; build-out \$245K–\$573K

Strategic read: The Joint spent 2025 deliberately shifting from company-owned to an asset-light franchisor model — refranchising 41 clinics and contracting to sell 22 more. The lesson for a roll-up is twofold: (1) the cash/membership model scales and throws off real EBITDA, and (2) the market rewards a clean, capital-light, recurring-revenue structure. It also signals that owning-and-operating at scale is operationally hard — which is exactly the value your MSO must add.

3.2 PE-backed corporate clinic groups (the true “roll-ups”)

- **Chiro One Wellness Centers (backed by The Vistria Group)** — roughly 120+ locations concentrated in the Midwest/West. The most prominent example of institutional PE capital backing an owned-and-operated chiropractic platform.
- **Champion Wellness Centers (recapitalized by Monument MicroCap Partners, March 2024)** — an active, textbook buy-and-build: add-on acquisitions of Brandon Spine & Injury Center (Feb 2025) and NuVita Chiropractic (2025). This is the model you would emulate at micro-cap scale.
- **MyoCore** — cited as an emerging institutional consolidator as the sector matures and draws more investor interest.
- **Insurer / retail entrants** — a PE-backed chiropractic firm has been contracted to pilot chiropractic services in ~15 Walmart locations with stated plans to scale toward 500, signaling that large retail and payer channels view chiropractic as a scalable MSK access point.

3.3 Franchise networks

- **HealthSource Chiropractic** — 300+ clinics across ~41 states; a franchise model combining chiropractic with progressive rehab.
- **The Joint (franchised)** — 885 franchised units; the dominant membership-retail franchise.

Franchising is a different animal from a PE roll-up: it scales the brand and collects royalties but does not capture the equity value of the underlying clinics. A roll-up that owns the EBITDA — not just licenses a brand — is what gets re-rated at exit.

3.4 The emerging structure of the market

The consensus forward view is a blended ecosystem: insurer-aligned MSK networks, PE-backed MSOs/roll-ups, hospital-affiliated clinics, and a still-large core of independents. The roll-up lane — owning and professionalizing clusters of clinics under one operating system — is the least crowded of the institutional plays and the one with the clearest multiple-arbitrage payoff.

4. The Deepest Problems in the Chiropractic Space

Every one of these problems is also an opportunity: they are the reasons independent offices underperform, and therefore the levers a centralized operator pulls to create the per-office uplift. They fall into six clusters.

4.1 Crushing overhead and thin, fragile margins

Independent practices in 2025 commonly run overhead above 55% of revenue, squeezed by rising payroll, rent, technology, and compliance costs. “Healthy” operating margins land in the 15–25% range, but many solo offices are sub-scale: the average sole-proprietor clinic does only ~\$140K in revenue against ~\$93K of expenses. Fixed costs (rent, front desk, equipment) do not shrink with low volume, so sub-scale offices are structurally unprofitable per provider-hour.

4.2 Reimbursement complexity and revenue leakage

Insurance is the single biggest operational drag. Roughly 80% of claims are estimated to contain errors, leading to denials, delayed cash, and write-offs. Reimbursement rates are flat-to-declining and vary by payer and treatment, and most owner-operators lack the billing sophistication to code correctly, appeal denials, and negotiate contracts. The money lost here is pure, recoverable EBITDA.

4.3 Patient acquisition and retention economics

Many clinics carry unsustainable patient-acquisition costs and have no systematic retention engine. Without memberships or care plans, revenue is episodic and unpredictable — patients come for an acute issue and disappear. The absence of recurring revenue both lowers lifetime value and makes the business look riskier to a buyer, depressing the multiple.

4.4 Owner-operator dependence (key-person risk)

In a typical office the owner-DC is the rainmaker, the lead clinician, and the manager. Revenue is tied to one person’s hands and relationships. This is the #1 reason small practices sell so cheaply: a buyer is purchasing a job, not a business. De-risking this — associate-driven care, systematized referrals, a brand bigger than any one DC — is the core of converting a “practice” into an “asset.”

4.5 Sub-scale back office and technology

Each independent office reinvents scheduling, billing, marketing, HR, compliance, and procurement on its own, usually on consumer-grade tools. There are no economies of scale, no purchasing power on supplies/equipment, and no data to benchmark performance across locations. Duplicated, sub-scale overhead is the most obvious synergy a platform removes.

4.6 Fragmentation and integration risk for acquirers

The flip side of cheap targets: practices have messy books, owner add-backs, inconsistent compliance, and care models built around one person. Diligence and post-close integration are where roll-ups succeed or fail. Saturation is also rising in adjacent verticals (dental, vet), where the best independents are already gone and prices have risen — a reminder to move with discipline while chiropractic is still early and cheap.

Problem	Why it persists	Roll-up solution
55%+ overhead	Sub-scale, no purchasing power	Shared services, vendor leverage
~80% of claims have errors	Owners aren't billing experts	Centralized RCM / billing team
Episodic revenue	No membership system	Recurring care-plan / membership model
Key-person dependence	Owner is the brand	Associate model, brand, systems
Duplicated back office	Each office is an island	Single MSO platform
Messy books	Owner-operator accounting	Standardized finance + reporting

5. The Operating Playbook — Where the “\$250K Per Office” Comes From

Dr. Wagner's claim that each office can add at least \$250K to earnings is credible — but only if it is decomposed into specific, stackable levers. Below is how that number is built, lever by lever. The figures are illustrative targets for a mature office doing, say, \$700K–\$1.2M of revenue; they should be re-run against each acquired clinic's actuals.

5.1 Revenue levers

Membership & recurring care plans

Converting episodic patients to monthly memberships or prepaid care plans is the highest-impact lever. It raises lifetime value, smooths cash flow, and — critically — makes the business look like recurring revenue to a buyer (worth a higher multiple). Membership conversion alone can add tens of thousands per office in net contribution and is the backbone of the model that made The Joint a public company.

Ancillary cash services

Layering cash-pay services onto the existing patient base is fast, high-margin revenue that does not depend on insurance: massage therapy, spinal decompression, cold laser, dry needling, shockwave therapy, and rehab. These also justify premium pricing and differentiate the clinic.

Retail & product

Supplements, orthotics, stabilizers, pillows, and topical products are repeat-purchase and high-margin. Industry data suggests dispensing stabilizers to even half of new patients can add up to ~\$50K/year per office. Supplement and nutrition programs are inherently subscription-friendly.

Pricing & collections

Many independents under-price and under-collect. Refreshing the fee schedule, enforcing point-of-service collection, and tightening the billing/denials process recovers revenue that is already being earned but not captured.

5.2 Cost levers

- **Centralized revenue-cycle management (RCM):** a single expert billing team cuts the ~80% claim-error problem, accelerates cash, and recovers denied revenue across all offices.
- **Shared back office:** one scheduling, HR, marketing, compliance, and finance function instead of duplicating it in every clinic.
- **Vendor & supply leverage:** group purchasing on equipment, supplements, malpractice, and software.
- **Marketing at scale:** centralized digital marketing and a recognized brand lower per-patient acquisition cost across the cluster.
- **Staffing model:** associate DCs and optimized provider scheduling raise revenue-per-square-foot and break the key-person dependence.

5.3 The per-office uplift, decomposed (illustrative)

Lever	Annual EBITDA uplift	Mechanism
Membership / care-plan conversion	\$60K–\$120K	Recurring revenue, higher LTV
Ancillary cash services	\$40K–\$80K	Massage, decompression, laser, rehab
Retail / supplements / orthotics	\$20K–\$50K	High-margin repeat purchases
Billing / RCM recovery	\$30K–\$70K	Fewer denials, faster, fuller collection
Overhead / purchasing savings	\$25K–\$60K	Shared services, vendor leverage
TOTAL per mature office	\$175K–\$380K	Stacked levers

Takeaway: a \$250K per-office uplift sits comfortably inside the achievable range and does not require every lever to fire at maximum. The discipline is in execution and in choosing offices with enough patient volume to support the added services.

6. Valuation & Multiple Framework — What Scale Wins the Top Multiple

This is the heart of the wealth creation. Value in a roll-up comes from two distinct engines: (1) growing EBITDA at each office, and (2) multiple arbitrage — the fact that the market pays a higher multiple for a large, professionalized, diversified platform than for a small, owner-dependent office. You must understand both.

6.1 What individual offices trade for

Valuation metric	Typical range (single office)	Notes
EBITDA multiple	~2.86x – 3.83x (often quoted 2–5x)	Small, owner-dependent practices
SDE multiple	~1.75x – 2.27x	Common for owner-operator clinics
Revenue multiple	~0.69x – 0.87x	Sanity-check metric

In plain terms: a solo office throwing off \$250K of true EBITDA is typically a ~\$750K–\$1M business at acquisition. That is your buy-side cost basis — and it is cheap.

6.2 What platforms trade for (the arbitrage)

As acquired offices are bundled into a professionally managed platform, the applicable multiple climbs. Across healthcare services the pattern is consistent:

Asset type	EBITDA multiple	Why
Single small office (add-on)	~3–5x	Owner-dependent, sub-scale, key-person risk
Sub-scale group / add-on	~4–7x	Some systems, still modest scale
Established platform	~8–10x+	Scale, clean books, recurring revenue, management depth
Premium / >\$5M EBITDA	+2 to +4 turns vs. add-ons	Diversification, de-risked, strategic buyer demand

The single most important valuation lever is scale of EBITDA. Crossing roughly \$5M of normalized, well-documented EBITDA is the threshold where a platform moves from “small add-on” pricing into institutional, 8–10x territory, because that is the size at which PE sponsors and strategics can underwrite it as a true platform. Getting to — and credibly past — \$5M EBITDA, with clean financials and a management team that is not you, is the goal that drives the top multiple.

6.3 The drivers that move you within the range

- **Recurring revenue mix** (memberships/care plans) — the biggest multiple booster.
- **Reduced key-person risk** — associate-driven care, professional management.
- **Clean, audited-quality financials** — GAAP-ish books, normalized add-backs, monthly reporting.

- **Revenue diversification** — cash + insurance + ancillary + retail, across multiple sites and markets.
- **Organic + de novo growth engine** — proof you can open new clinics, not just buy them.
- **Geographic density** — clustered markets allow shared management and brand efficiency.

7. The Roll-Up Math — A Worked Model

The following illustrative model shows how the three engines compound. It uses round numbers to make the mechanics transparent; the real model should be built off Dr. Wagner's target list and actual P&Ls. Assume a cluster of 12 acquired offices, each ~\$250K EBITDA at acquisition, plus the operating uplift.

7.1 Step 1 — Acquire cheap

Acquisition assumptions	Value
Offices acquired	12
EBITDA per office at purchase	\$250K
Aggregate acquired EBITDA	\$3.0M
Blended purchase multiple	4.0x
Total purchase price (enterprise)	\$12.0M

7.2 Step 2 — Lift EBITDA per office

Operating uplift	Value
Uplift per office (target)	\$250K
Post-uplift EBITDA per office	\$500K
Aggregate office EBITDA	\$6.0M
Less: MSO / corporate overhead	(\$1.0M)
Platform normalized EBITDA	\$5.0M
Effective post-synergy purchase multiple	2.4x (\$12.0M / \$5.0M)

7.3 Step 3 — Re-rate at exit

Exit scenario	Multiple	Enterprise value
Conservative	8.0x	\$40.0M
Base case	9.0x	\$45.0M
Premium (scale + recurring rev.)	10.0x	\$50.0M

Value creation bridge: \$12.0M invested into EBITDA → grown to \$5.0M of platform EBITDA → exited at ~\$45M base case. That is roughly \$33M of enterprise value created (before debt and fees), or about

3.75x the enterprise capital deployed — and materially higher on equity if acquisitions are part-financed with debt. The arbitrage (buying at 4x, exiting at 9x) does roughly half the work; the operating uplift does the other half.

Note on leverage: if ~50% of each acquisition is debt-financed, the equity multiple roughly doubles relative to the all-equity case, because debt is repaid from clinic cash flow while equity captures the full re-rating. Leverage amplifies returns — and risk — and should be sized to the stability of the recurring-revenue base.

8. Risks & Watch-Outs

- **Corporate-practice-of-medicine / ownership rules:** many states restrict non-licensee ownership of clinical practices. This is exactly why the MSO/management-services structure exists — the DC-owned PC delivers care; the MSO (which you own) provides management services under a contract. Get this structured by healthcare counsel before the first close.
- **Reimbursement pressure:** flat-to-declining insurance rates make a cash/membership/ancillary mix strategically important, not just nice-to-have.
- **Integration risk:** messy books and owner add-backs make diligence and normalization essential; over-paying for inflated SDE is the classic roll-up mistake.
- **Key-person transition:** retain selling DCs with earn-outs/equity and build associate capacity so revenue survives the founder's exit.
- **Multiple-compression / saturation:** healthcare roll-up returns are under more scrutiny than a few years ago; the edge goes to operators who actually improve clinics, not just stack them. Buy disciplined, integrate well.
- **Capital & financing:** a credible exit multiple assumes clean financials and audited-quality reporting from day one — build the finance function early.

9. Recommended Roadmap to the Top Multiple

1. Structure first: stand up the MSO + PC structure with healthcare counsel; design the management services agreement, compliance, and reporting backbone.
2. Acquire an anchor: buy 1-2 strong, profitable offices in a target metro as the platform anchor and proving ground for the operating model.
3. Prove the playbook: implement memberships, ancillary services, centralized RCM, and shared marketing on the anchor; document the per-office uplift with real numbers.
4. Cluster and scale: acquire 8-15 offices in contiguous markets at 3-5x; integrate each onto the single MSO platform; layer in de novo openings to show organic growth.
5. Institutionalize: build management depth so the business does not depend on you or any one DC; produce monthly, audit-ready financials; track unit economics by clinic.

6. Cross \$5M EBITDA: reach the scale and recurring-revenue mix that justify 8–10x; clean up the cap table and diligence file.
7. Exit or recapitalize: sell to a PE sponsor or strategic, or take a recap that returns capital while retaining upside for the next leg of growth.

Bottom line: the chiropractic market is large, cheap, and barely consolidated; the per-office uplift Dr. Wagner describes is achievable through well-known operating levers; and the real prize is the multiple arbitrage that rewards getting past ~\$5M of clean, recurring-revenue-rich EBITDA. Buy disciplined, operate hard, document everything, and structure for the exit from day one.

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All financial figures are industry benchmarks and illustrative ranges intended to frame the strategy. Before committing capital, confirm every number against the actual financials of each target practice and obtain healthcare-specific legal and tax counsel on MSO structuring.



ChiroPillar
STRENGTH IN ALIGNMENT

ChiroPillar Digital

Back-Pain & Telehealth App — Product, Monetization & Insurance Deep Dive

Prepared for Eric & Dr. Wagner · KingdomBroker PE Fund I · June 2026

Free → \$9/month → \$74/year, with telehealth and local doctor/chiropractor matching

Contents

1. Executive Summary

ChiroPillar Digital is a free-to-start mobile app that helps people understand and manage their back (and broader musculoskeletal) pain with an AI-guided assessment and exercise program — and, when they need a human, matches them to a telehealth visit or a local ChiroPillar chiropractor/physician. It is the consumer front door to the clinic roll-up: a low-cost, high-scale engine that captures pain sufferers early, builds trust through genuinely useful free care, and converts a meaningful share into recurring app subscribers and, more valuably, into paying in-clinic patients.

The strategic logic is that the app and the clinics make each other worth more. Pure-digital MSK companies (Hinge Health, Sword Health) are worth billions but cannot deliver hands-on adjustment or in-person care; independent clinics have the hands-on care but no scalable digital acquisition funnel or recurring digital revenue. ChiroPillar can own both — a defensible position neither side can easily copy.

Three revenue layers: (1) app subscriptions (Free → \$9/mo → \$74/yr) as recurring MRR; (2) reimbursable clinical services — telehealth consults and remote therapeutic monitoring (RTM) billed through the clinics/providers; and (3) the highest-value layer, in-clinic patient conversion, where an app user who books a ChiroPillar visit becomes a multi-hundred-to-thousand-dollar lifetime patient.

Bottom line: the app is cheap to scale, improves clinical outcomes (AI exercise programs show 50–68% pain reduction in studies), creates recurring revenue, and — critically — feeds patients into the clinics that drive the platform's enterprise value and exit multiple.

2. Market Opportunity

Back pain is one of the most common and costly health complaints in the world, and the digital tools to manage it are growing fast. The digital musculoskeletal (MSK) care market is estimated at roughly \$5–10B in 2025 and is projected to reach ~\$11.6–16B by 2030–2032, growing ~17–18% per year. Online/virtual consultations are already the largest segment, and the shift toward telehealth, remote monitoring, and value-based care is accelerating.

The category has produced large winners, which both validates the model and shows the gap we can exploit:

Player	Scale / value (2025)	Model & gap
Hinge Health	~\$3B market cap at May 2025 IPO; FY25 free cash flow ~\$180M	Sells to self-insured employers; engagement-based pricing. No in-person/hands-on care.
Sword Health	~\$4B valuation (June 2025)	Therapist-led, sensor-based digital PT; acquired Kaia Health for \$285M. Employer/payer channel; no clinics.
Kaia Health	Acquired by Sword (\$285M)	Camera/computer-vision exercise coaching. Now absorbed.
ChiroPillar Digital	New entrant	Direct-to-consumer freemium + telehealth + owned local clinics. The hybrid the incumbents can't easily

Player	Scale / value (2025)	Model & gap
		build.

The wedge: incumbents win the employer contract but hand the patient nothing local when they need real hands-on treatment. ChiroPillar starts with the consumer directly, is free to begin, and can escalate to a telehealth visit or a nearby ChiroPillar clinic — closing the loop from app to adjustment table.

3. The Product

A simple, friendly app (fronted by the ChiroPillar brand and mascot) that meets people at the moment their back hurts and walks them from self-help to professional care as needed.

3.1 Core flow

1. Assess — a short AI-guided intake screens symptoms, flags red flags (e.g., signs needing urgent medical review), and classifies the likely problem.
2. Guide — a personalized daily exercise and education program adapts as the user logs pain and progress.
3. Connect — one tap to a telehealth consult or to book an in-person visit with a local ChiroPillar chiropractor or partner physician.
4. Monitor — for appropriate patients, remote therapeutic monitoring (RTM) tracks adherence and outcomes between visits — and is reimbursable (see Section 6).

3.2 Pricing tiers

Tier	Price	What's included
Free	\$0	AI back assessment, red-flag screening, starter exercise library, pain/progress tracking, educational content, clinic locator.
Plus (monthly)	\$9 / mo	Full personalized AI program, unlimited exercises & content, in-app messaging, progress insights, discounted telehealth, priority clinic booking.
Plus (annual)	\$74 / yr	Everything in Plus at ~31% off vs. monthly (best value; improves retention & cash flow).
Clinical add-ons	Billed / cash	Telehealth consults and RTM via providers (often insurance-reimbursable); in-clinic care via ChiroPillar locations.

Why this structure: Free removes all friction and maximizes top-of-funnel; \$9/mo is an easy consumer price point for a daily-use health tool; the \$74 annual plan (vs. \$108 at monthly) pulls users into a sticky, prepaid year that smooths cash flow and lifts lifetime value. The clinical layer is where insurance and high-value patient relationships enter.

4. AI & Clinical Evidence

The AI does three jobs: triage (classify the problem and screen for red flags), personalization (compose and adapt an exercise program to the individual), and escalation (decide when a human clinician should step in). This keeps the free tier genuinely helpful while routing the right patients to billable care.

The evidence base for app-based, AI-guided exercise in back pain is encouraging. Published studies of digital exercise programs report average pain reductions in the range of roughly 50–68% over 8–12 weeks, with a large cohort showing ~68% average pain improvement and ~79% of completers achieving a clinically meaningful change; controlled studies show reduced disability scores versus standard group therapy. This matters commercially as well as clinically: better outcomes drive retention, word-of-mouth, and credibility with payers.

Guardrails: the AI is a triage and coaching tool, not a diagnostician. Red-flag screening must escalate to a licensed provider; all clinical decisions and any billable services run through credentialed chiropractors/physicians. This protects patients and keeps the model compliant.

5. Monetization & Unit Economics

Three stacked revenue engines, in order of scale vs. value:

- **Subscriptions (high volume, modest ARPU):** recurring MRR from Plus users.
- **Reimbursable clinical services (mid volume, real margin):** telehealth + RTM billed through providers.
- **In-clinic conversion (lower volume, highest value):** app users who become paying ChiroPillar patients — the real prize.

5.1 Conversion benchmarks

Freemium apps typically convert ~2–5% of free users to paid; health & fitness apps that use a free trial convert far higher (median ~40%, top performers ~68%). A blended funnel using a generous free tier plus targeted trial offers can realistically target the mid-single-digits to low-teens on paid conversion as the product matures.

5.2 Illustrative MRR model (Year 1, conservative)

Assumption / output	Value
Free downloads (Year 1)	100,000
Paid conversion	4% → 4,000 subscribers
Blended ARPU	~\$8 / month
Monthly recurring revenue (MRR)	~\$32,000
Subscription ARR	~\$384,000
App users booking a clinic visit (3%)	~3,000 patients
Avg patient value to clinic (illustrative)	\$800–\$1,500

Assumption / output	Value
Clinic revenue influenced by app	~\$2.4M–\$4.5M

Read this carefully: the subscription ARR (~\$384K) is nice recurring revenue, but the in-clinic conversion line (millions) is where the app earns its keep. The app's true ROI is as the cheapest, most scalable patient-acquisition channel the clinics have — every dollar of clinic EBITDA it creates is then re-rated at the platform's exit multiple (see the roll-up deep dive).

6. Insurance & Reimbursement — “Can Insurance Cover It?”

Yes, in specific, well-defined ways — but it flows through licensed providers and clinics, not the consumer app subscription itself. The three reimbursable pathways:

6.1 Remote Therapeutic Monitoring (RTM)

RTM is the key reimbursement vehicle for an exercise/adherence app tied to musculoskeletal care. CMS established RTM CPT codes that providers (including physical therapists, and physicians) can bill under Medicare and many commercial plans:

CPT code	Approx. reimbursement	What it covers
98975	~\$20 (one-time)	Initial setup & patient education (requires ≥2 days monitoring).
98977	Device/data code	MSK monitoring data supply, 16–30 days in a 30-day period.
98980 / 98981	~\$50 / ~\$40	Treatment management, first 20 min / each additional 20 min per month.

CMS's 2026 final rule expands RTM further (new codes for shorter monitoring periods), improving the economics for MSK programs. Practically: an app-monitored patient under a ChiroPillar provider can generate recurring monthly RTM billing on top of (or instead of) the cash subscription.

6.2 Telehealth visits

Telehealth consults with a provider are reimbursable. Medicare covers many telehealth services (current flexibilities extended into 2027 for non-behavioral care in the home), and as of 2025, 44 states plus DC have private-payer telehealth laws, with ~24 states requiring payment parity (same rate as in-person). Note the chiropractic nuance: Medicare itself only pays chiropractors for manual spinal manipulation (which is inherently in-person), so telehealth billing for the app's consults is best routed through physicians/PTs or via commercial plans and cash, with chiropractic adjustments delivered in-clinic.

6.3 Cash & membership (the dependable base)

Where insurance doesn't reach, the \$9/\$74 subscription and clinic cash memberships cover it. This blended model — reimbursable where possible, cash where not — mirrors what already works in modern chiropractic and avoids over-dependence on any single payer.

Net: market the consumer app as affordable cash (\$0/\$9/\$74); layer insurance-reimbursable RTM and telehealth for clinically appropriate patients through the providers. Build RTM billing and telehealth compliance into the clinic operations from day one.

7. Go-to-Market

- **Consumer-direct:** app-store optimization, content/SEO on back-pain topics, social, and the ChiroPillar brand/mascot for memorability.
- **Clinic cross-sell:** every ChiroPillar patient is offered the app for between-visit care and RTM — instant, free distribution and higher retention.
- **Local geo-targeting:** concentrate app marketing where ChiroPillar clinics exist so digital demand converts to in-person bookings.
- **Employer/payer (later):** once outcomes data accrues, pursue the same self-insured-employer channel Hinge/Sword use — but with the differentiator of local in-person care.

8. Risks & Moats

Risks

- **Regulatory:** state telehealth and licensure rules, RTM billing compliance, and corporate-practice-of-medicine constraints (use the MSO/PC structure from the roll-up plan).
- **Clinical liability:** red-flag mis-triage; mitigate with conservative escalation and licensed-provider oversight.
- **Churn:** health-app engagement decays; combat with habit features, human touchpoints, and the annual plan.
- **Competition & CAC:** well-funded incumbents; avoid head-to-head on the employer channel early and win locally first.

Moats

- **Owned in-person network:** the one thing Hinge/Sword can't replicate quickly — local hands-on care attached to the app.
- **Data flywheel:** proprietary outcomes data across digital + in-clinic improves the AI and supports payer contracts.
- **Brand + recurring revenue:** ChiroPillar brand trust plus subscription MRR raise both retention and the eventual platform multiple.

9. Roadmap (Phased)

5. MVP (0–6 mo): free assessment + exercise library + pain tracking + clinic locator; launch in ChiroPillar markets; instrument analytics.
6. Monetize (6–12 mo): introduce \$9/\$74 Plus tiers and in-app telehealth booking; begin RTM billing through clinics.

7. Personalize (12–18 mo): mature the AI program engine; add messaging and outcomes dashboards; optimize conversion funnel.
8. Scale (18–36 mo): expand alongside clinic acquisitions; accumulate outcomes evidence; pursue employer/payer contracts.

Strategic takeaway: build the app as the patient-acquisition and recurring-revenue front end of ChiroPillar. Keep it free to start, genuinely useful, and tightly wired to the clinics — so it both stands alone as a digital-health asset and multiplies the value of the roll-up.

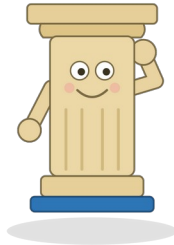
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All market figures, conversion rates, reimbursement amounts, and the MRR model are illustrative industry benchmarks for planning. Confirm current CPT reimbursement, state telehealth/licensure rules, and chiropractic billing limits with healthcare counsel and a billing specialist before launch. Not legal, medical, or financial advice.

ChiroPillar

Brand & Mascot Brief



ChiroPillar

STRENGTH IN ALIGNMENT

Prepared for Eric & Dr. Wagner • KingdomBroker PE Fund I • June 2026

1. Brand at a glance

Name: ChiroPillar (chiroprillar.com)

Idea: A standardized, national chiropractic platform built by rolling up independent offices under one trusted brand. The name fuses *Chiro* (spine) + *Pillar* (a strong, upright support) — the spine is your pillar.

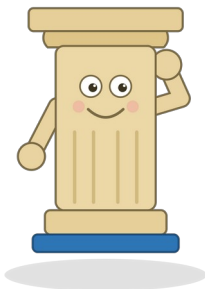
Personality: Strong, supportive, reassuring, approachable. Confident without being clinical or intimidating.

Tagline options: “Strength in alignment” (current) • “Built to support you” • “Stand tall” • “Your foundation for life.”

2. The mascots

Two characters, two jobs. Cap is the public-facing personality that sells the name; the Atlas vertebra is a meaningful supporting device for clinical and educational use. You can run both.

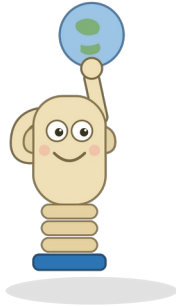
Cap — the friendly pillar (primary)



A personified column standing tall and strong, with the shaft’s flutes doubling as a stylized spine.

- **Why it wins:** it IS the word “ChiroPillar” — instant recognition, zero explanation.
- **Use for:** signage, ads, website, kids/family touchpoints, merch.
- **Watch-out:** keep the flex pose light/friendly so it reads supportive, not macho.

Atlas — the vertebra (supporting symbol)



The atlas (C1) vertebra personified, balancing the “world” effortlessly on a small stack of vertebrae.

- **Why it works:** anatomically true (atlas = top vertebra), gender-neutral, inclusive — balance, not brawn.
- **Use for:** patient education, “meet your spine” content, app icon, clinical materials.
- **Note:** needs one sentence of story the first time — great for onboarding.

3. Color palette

Swatch	Name / Hex	Where to use
	Deep Spine Blue #1F4E79	Primary brand color — wordmark, headers, base plinth.
	Align Blue #2E75B6	Secondary/accent — “Chiro” in wordmark, buttons, highlights.
	Column Stone #EBD8A6	Mascot body / warm neutral — friendly, trustworthy.
	Stone Outline #7A6A45	Outlines and grounding detail.
	Globe Blue #9CC4E4	Light accent — Atlas globe, soft backgrounds.
	Cheek Coral #F2B0A0	Tiny warm accent only — cheeks, small highlights.
	Neutral Gray #6B7785	Taglines, secondary text, captions.

4. Do's and don'ts

Do

- Keep the mascot warm, rounded, and approachable — friendly to seniors, women, kids, and athletes alike.
- Lead with “support, stability, alignment” — the brand carries the load for you.
- Use clear space around the logo (min. one capital-height on all sides) and keep the blue base on the mascot.
- Reserve the Atlas vertebra for educational/clinical moments where the spine story adds meaning.

Don't

- **Don't use a literal caterpillar mascot or lean on the "cater-pillar" pun.** Caterpillar Inc. is one of the most aggressive trademark enforcers in the U.S. (it has pursued tiny unrelated businesses over just "Cat" and won famous-mark dilution cases). A caterpillar + "ChiroPillar" invites an opposition. Avoid the bug entirely.
- Don't over-muscle the mascot — no bodybuilder physique; strength should look effortless.
- Don't stretch, recolor outside the palette, or place the logo on busy backgrounds.
- Don't finalize the identity before trademark + domain clearance (see section 6).

5. Clinic naming & signage lockup

Roll-up offices adopt a consistent endorsed-brand format so every acquired clinic reads as one national platform while keeping a local feel:

ChiroPillar — [City / Neighborhood]

Examples: ChiroPillar — Brandon • ChiroPillar — Tampa Heights • ChiroPillar — Plano. Mascot "Cap" appears on storefront glass and reception; the Atlas vertebra appears on intake/education materials. App icon uses the simplified capital-and-face mark.



App / favicon mark

6. Clear it before you build (important)

- Run "ChiroPillar" through the USPTO trademark search in Class 044 (medical/healthcare services) — our web check found no existing chiropractic brand using it, but confirm formally.
- Lock the exact chropillar.com (and common variants/social handles); budget to acquire the .com if it's parked.
- Have healthcare-IP counsel review the name + mascot combination, especially given the Caterpillar caution above.
- File a trademark for the wordmark and the chosen mascot mark once cleared.

7. Files in this pack

- chropillar_logo_lockup — primary horizontal logo (JPEG + PNG + SVG)
- cap_mascot — Cap the pillar, standalone (JPEG + PNG + SVG)
- atlas_mascot — Atlas vertebra, standalone (JPEG + PNG + SVG)

- chiropillar_app_icon — app/favicon mark (JPEG + PNG + SVG)

SVG files are the master/scalable source — hand these to a designer for vector refinement. PNGs are transparent; JPEGs have a white background for quick drop-in. These are concept directions to brief a professional designer, not final production logos.